@apaji_x ADHD claims: evidence check

Date checked: 11 Oct 2026 (re-evaluated 11 Oct 2026 with the Scientific Evidence Evaluator framework)
Scope: posts from roughly 10 Oct 2026, 1:59 AM–9:14 PM IST (58 original posts; 12 self-reply chains/standalones referenced; 15 reposts not checked)

2 Supported	6 Partially supported	4 Insufficient evidence	6 Unsupported

Section 1 · Claims

Supported 2

Claim #6 · Supported · High confidence

Research Quality: Claim: in ADHD, emotions hit faster and harder and take longer to settle. Two Level 1 meta-analyses (GRADE moderate, strong consensus) show large effects in adults (g=1.17) [1] and youth (d≈0.95, n=32,044) [2]. "You can't stop them" overreaches, because emotion regulation responds to treatment. Red flags: funnel-plot asymmetry in Beheshti (possible publication bias), and mostly self-report measures; pre-registered studies using lab or ecological measures would strengthen it.

X Post Link · X Post Link 2 · Citations for this claim

Brain scans proved why emotions hit at 100% for people with ADHD, and you can't stop them. You are not too sensitive or dramatic: 1/4

1/4 – Brain scans proved why emotions hit at 100% for people with ADHD, and you can't stop them. You are not too sensitive or dramatic: 1/4

2/4 – Your brain lacks the filter that others have.
One comment, someone forgetting to text back, or even a weird look can hit you all at once. People have probably told you forever that you overreact, need to toughen up, or make things a bigger deal than they are.

But brain scans show your amygdala fires like a real threat is in front of you. You cannot simply decide to feel less. Trying to suppress it only makes the pressure build until you break down or lash out, and the shame cycle begins again.

3/4 – Use a “Sensory Shock Switch” when the wave hits.

As soon as you feel that emotional wave coming, grab an ice cube and squeeze it in your hand, or splash your face with cold water. Just focus on how cold it feels for thirty seconds.

This jolts your nervous system from emotional overload into physical awareness. It breaks the hijack long enough for your rational brain to come back online.

The pain worsens when you try to think your way out. The switch gives your brain an off-ramp.

4/4 – Brain scans show that ADHD RSD lights up the same pain centers as getting physically hurt. Your feelings aren't over the top.

They are neurologically amplified. This means that what feels like a ten to you may register as a two to someone else, not because you are weak, but because your brain processes it differently.

ALSO CITED: STOP CALLING YOURSELF "TOO SENSITIVE." THAT UNANSWERED TEXT DIDN'T HURT BECAUSE YOU'RE DRAMATIC. YOUR ADHD BRAIN FEELS REJECTION AT A DIFFERENT VOLUME.

1/6 – Stop calling yourself "too sensitive." That unanswered text didn't hurt because you're dramatic. Your ADHD brain feels rejection at a different volume.

They replied "ok." No emoji, no exclamation mark. Now you've spent two hours rewriting everything you might have done wrong. That's rejection sensitivity.

2/6 – 1. Rejection sensitive dysphoria (RSD) is an intense emotional pain triggered by perceived rejection, criticism, or disappointing someone. The key word is "perceived." It can hit even when nobody actually rejected you.

3/6 – 2. It's not weakness. Emotional dysregulation is common in ADHD, so feelings arrive faster and stronger, and they take longer to settle. Many people with ADHD also spend years hearing criticism, which can make the brain braced for it.

4/6 – 3. Common signs: replaying conversations for hours, assuming silence means anger, people-pleasing to avoid disapproval, avoiding things you might fail at, or going from fine to devastated in seconds over a tone or a look.

5/6 – 4. Pause before you react. When the wave hits, wait 20 minutes before replying, quitting, or apologizing. The first story your brain tells is usually the harshest one, and the feeling drops faster than you'd expect.

6/6 – 5. Check the story, not just the feeling. Ask: "What do I actually know?" versus "What am I assuming?" Then tell the people close to you how it works, like "If I seem distant after feedback, I'm processing, not angry." It takes pressure off both sides.

Claim #15 · **Supported** · High confidence

Research Quality: Claim: B12 deficiency can cause fatigue, memory and concentration problems, and nerve damage. This is established clinical knowledge from case series and treatment response (GRADE moderate, strong consensus) [1]. No red flags. The claim is not ADHD-specific, and we found no B12–ADHD link.

X Post Link · Citations for this claim

Vitamin B12 deficiency can affect your brain more than you realize. 🧠Low levels may contribute to fatigue, memory problems, poor concentration, and nerve damage.

1/1 – Vitamin B12 deficiency can affect your brain more than you realize. 🧠Low levels may contribute to fatigue, memory problems, poor concentration, and nerve damage.

Your body needs B12 to maintain healthy nerves and produce red blood cells, yet its deficiency can go unnoticed. https://t.co/jK4ckIyLkX

> **Quoted post (another account) –** BREAKING: Scientists confirm that vitamin B12 deficiency can directly impair the brain's ability to function normally — because B12 is essential for nerve health and red blood cell formation.
>
> Low B12 can contribute to fatigue, memory problems, and difficulty concentrating. ⬇️⬇️

Partially supported 6

Claim #3 · **Partially supported** · Moderate confidence

Research Quality: Claim: anxiety, borderline traits and autistic burnout can mimic ADHD, with causes the post asserts (sensory rather than fear-based restlessness, masking). A systematic review (Level 1, GRADE low, moderate consensus) supports the overlap, especially with borderline-level emotion dysregulation [1]. The causal mechanisms are untested. Red flags: mostly small cross-sectional studies and no pooled estimate; diagnostic-accuracy studies with structured interviews would strengthen it.

X Post Link · Citations for this claim

YOUR THERAPIST THINKS YOU'RE DEPRESSED.

1/4 – YOUR THERAPIST THINKS YOU'RE DEPRESSED.

You might be in ADHD burnout. They look identical from the outside. They need opposite treatments. Getting it wrong costs years. 1/4

2/4 – 1. Depression says you cannot feel joy. ADHD burnout says you can feel joy, but you cannot start, choose, or finish. Both bring exhaustion, sleep chaos, irritability, and hopelessness.

The difference lives in the pattern. With depression, energy often returns after rest. With ADHD burnout, rest does not restore you because your brain is still running twelve tabs and masking every one. 2/4

3/4 – 2. Anxiety can look like ADHD: racing thoughts, restlessness, doom. But ADHD restlessness is often sensory or understimulation, not fear. Borderline traits can look like rejection sensitivity and mood storms. Autistic burnout can look like ADHD shutdown, but the cause is masking and sensory overload.

Misdiagnosis happens when clinicians treat the visible emotion, not the nervous system below. 3/4

4/4 – 3. Standard CBT can fail when the problem is executive function, not distorted thinking. Antidepressants can fail when the problem is dopamine, not serotonin. Relief comes from accurate recognition: ADHD coaching, medication, sensory accommodation, nervous system safety, and grief for lost years.

You are not lazy, broken, or treatment resistant. You were described with the wrong map. 4/4

Claim #5 · Partially supported · Moderate confidence

Research Quality: Claim: shame floods the prefrontal cortex with cortisol and impairs executive function, so self-compassion restores it. Level 1 meta-analyses in the general population (GRADE low, emerging consensus) show social-evaluative stress raises cortisol [1] and slightly impairs working memory (g=−0.20) [2], and animal work links stress to prefrontal impairment [3]. However, cortisol alone does not reproduce these effects, and the shame → self-compassion chain has never been tested in ADHD. RCTs of self-compassion in ADHD, measuring cortisol and executive function, would strengthen it.

X Post Link · Citations for this claim

BREAKING: Neuroscientists confirm that shame is the number 1 hidden symptom of ADHD.

1/6 – BREAKING: Neuroscientists confirm that shame is the number 1 hidden symptom of ADHD.

Not hyperactivity. Not distraction.

Shame.

From a lifetime of being told you're not trying hard enough. 1/6

2/6 – 1. Every person with undiagnosed ADHD carries a running list of evidence that they are fundamentally not enough. The forgotten things. The unfinished things. The broken promises. The wasted opportunities. The list has been building since childhood and they believe every single item on it. 2/6

3/6 – 2. The shame is not a side effect of ADHD. It is a symptom. It is what happens when a neurological condition goes unnamed and every manifestation of it gets explained as a character flaw instead. You are not lazy. But you have been told you are lazy for so long that the difference stopped mattering. 3/6

4/6 – 3. Shame in ADHD compounds everything. It is not just an emotion. It is a neurological problem. Shame floods the prefrontal cortex with cortisol. The prefrontal cortex is also where executive function lives. Shame actively destroys the function it is blaming you for not having. 4/6

5/6 – 4. This is why self compassion is not soft. For someone with ADHD, reducing shame is neurologically strategic. Less shame means less cortisol. Less cortisol means better prefrontal access. Better prefrontal access means better executive function. Kindness toward yourself is literally functional. 5/6

6/6 – 5. The shame was never accurate. It was the only explanation available when the real one was missing. Now there is a real one. And the real one has nothing to do with effort and everything to do with neurology. 6/6

Claim #9 · Partially supported · Moderate confidence

Research Quality: Claim: wanting and starting run on different systems, the starting side runs low in ADHD, and dopamine responds only to novelty or urgency. Reward differences are supported by Level 1 imaging meta-analyses and PET studies [1] [2] [3] [4] (GRADE low, moderate consensus). However, dopamine-transporter findings are inconsistent and confounded by prior stimulant use [5], and "dopamine ignores

goals" is unsupported. Red flags: small PET and fMRI samples, a reused Volkow sample, and a published erratum; larger pre-registered imaging studies separating medication effects would strengthen it.

X Post Link · X Post Link 2 · Citations for this claim

THE CRUELEST PART OF ADHD ISN'T THE BORING STUFF. IT'S THE GUITAR IN THE CORNER THAT NEVER GETS PLAYED.

1/7 – THE CRUELEST PART OF ADHD ISN'T THE BORING STUFF. IT'S THE GUITAR IN THE CORNER THAT NEVER GETS PLAYED.

Wanting to play it and being able to start run on two different systems. 1/7

2/7 – You've wanted to draw all week. Saturday comes. The sketchbook is right there on the table.
You pick up your phone instead, and two hours disappear.
And the worst part isn't the two hours.

3/7 – It's the voice that says: if you really loved it, you'd do it.
Neuroscientist Kent Berridge spent decades showing that liking something and wanting to go do it run on different brain systems. With ADHD, the starting side often runs low even when the liking side is loud.
You still love it. The engine just won't turn over.

4/7 – Your phone wins every time because it costs zero steps.
The guitar costs five. Find the pick, tune it, pick a song, sit down, decide you're ready. An ADHD brain counts every single one.
Fun with setup loses to fun without it.

5/7 – So get the steps down to zero the night before. Guitar out of the case and on the couch. Paints open on the table. The game already loaded.
And borrow someone's momentum: "I'm drawing for ten minutes, want to sit with me?" It's called body doubling, and it works on fun stuff too.

6/7 – Meanwhile the hobby graveyard grows. The half-built LEGO set. The paints still in plastic. The puzzle that never left the box.
Someone you love asks, "Why do you keep getting stuff you never use?" You don't have an answer. So you stop getting it. Then you stop calling yourself someone who draws.
What's in your hobby graveyard?

7/7 – So "if you really loved it, you'd do it" was never the test. You loved it the whole time. Starting was the broken part.
The guitar still won't play itself, and the people around you get to wonder. But the answer was never more love. It was fewer steps.

ALSO CITED: ADHD IS LIKE: I COULD START A BUSINESS. I COULD WRITE THE BOOK. I COULD COMPLETELY REINVENT MY LIFE BY NEXT MONTH.

1/4 – ADHD IS LIKE: I COULD START A BUSINESS. I COULD WRITE THE BOOK. I COULD COMPLETELY REINVENT MY LIFE BY NEXT MONTH.

And also like: replying to one email is too many steps. Pass. 1/4

2/4 – The grand vision costs me nothing. Imagining the business, the book, the whole reinvented life fires dopamine instantly, because it lives in the abstract, where nothing can fail yet.

The email lives in the concrete. Concrete means steps. Steps mean friction I cannot pay for.

So I am not inconsistent. I am running two completely different nervous systems. 2/4

3/4 – The gap is not discipline. Initiation is its own neurological function, separate from desire. I can want something with my whole chest and still have no bridge to the first movement.

My nervous system is not lazy. It is waiting for a spark it cannot manufacture on command. 3/4

4/4 – Dopamine does not arrive because something matters to me. It arrives because something is novel, urgent, or witnessed by another person. That is the entire mechanic, and it has never once cared about my goals.

So this was never weak will. It is a nervous system that never receives the signal to move, no matter how badly the dream is wanted. 4/4

Claim #12 · **Partially supported** · Moderate confidence

Research Quality: Claim: ADHD working memory can't hold multi-step spoken instructions, so written instructions remove a barrier. A Level 1 meta-analysis (38 studies) supports a moderate working-memory deficit [1], but the accommodation itself has never been tested (GRADE low). Red flags: group averages with wide individual variation, and bias tests that couldn't be checked (no free full text). A trial comparing written and spoken instructions would strengthen it.

X Post Link · Citations for this claim

ACTUALLY, YOU ARE ALLOWED TO ASK FOR ADHD WORKPLACE ACCOMMODATIONS...

1/7 – ACTUALLY, YOU ARE ALLOWED TO ASK FOR ADHD WORKPLACE ACCOMMODATIONS...
WITHOUT EVER SAYING THE WORD ADHD.

Most people with ADHD never ask. They just burn out quietly instead. 1/7

2/7 – 1. Most of these don't need a diagnosis on the table.

"I do my best work with written follow-ups." "I focus better with headphones on." Framed as working preferences, a lot of these get a yes and nobody asks why. Formal accommodations through HR can ask for paperwork. The everyday ones usually don't.
You do not owe anyone your full neurological history to get the conditions that let you do your job well.

3/7 – 2. Written instructions are a normal thing to ask for.

Your manager rattles off six steps in the hallway. By step four, step one is gone. ADHD working memory can't reliably hold a multi-step verbal briefing.
Asking for it in writing is not laziness. It's removing a barrier a neurotypical colleague never has to deal with. One line does it: "Can you drop that in a message so I get every step right?"

4/7 – 3. So are headphones, or a quieter desk.

Open-plan noise is a documented ADHD barrier. Every conversation two desks over pulls attention off the work.
This is not a preference. It's a reasonable adjustment, and it can be the difference between functional and non-functional.

5/7 – 4. Someone will call it asking for special favors.

But an accommodation is not an advantage. It is the removal of a disadvantage.
A ramp isn't a special favor for a wheelchair user. It's equal access. ADHD accommodations work exactly the same way.

6/7 – 5. Without them, the job costs double.

The harder the work, the more these matter. Without them, half your energy goes into fighting the environment instead of doing the work.
That gap is where burnout comes from.

7/7 – So the headphones aren't a favor, and the written recap isn't a weakness. They're what lets the work you're actually good at show up.
Some managers will still roll their eyes. And asking can still feel like admitting you can't keep up. It isn't. It's the reason you can.

Claim #13 · **Partially supported** · Moderate confidence

Research Quality: Claim: open-plan noise is a documented ADHD barrier, and headphones or a quiet desk help. A pre-registered Level 1 meta-analysis found a small benefit of white or pink noise in ADHD (g=0.25, no publication bias) [1], but results vary by symptom profile [2], and the evidence is lab-based and mostly in children (GRADE low, emerging consensus). Red flags: indirectness to adult offices, and the noise-benefit hypothesis comes from Söderlund's own group. Workplace studies of adults with ADHD would strengthen it.

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Claim #17 · **Partially supported** · Moderate confidence

Research Quality: Claim: suppressing emotions in ADHD builds pressure until a breakdown or outburst. Adults with ADHD habitually suppress more [1], and in a small randomised lab experiment (n=36) suppression prolonged distress compared with acceptance [2] (GRADE low, emerging consensus). Red flags: small samples, no non-ADHD comparison group, and "break down or lash out" was not measured. Ecological momentary studies linking suppression to later outbursts would strengthen it.

X Post Link · Citations for this claim

Brain scans proved why emotions hit at 100% for people with ADHD, and you can't stop them. You are not too sensitive or dramatic: 1/4

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probably told you forever that you overreact, need to toughen up, or make things a bigger deal than they are.

But brain scans show your amygdala fires like a real threat is in front of you. You cannot simply decide to feel less. Trying to suppress it only makes the pressure build until you break down or lash out, and the shame cycle begins again.

3/4 – Use a “Sensory Shock Switch” when the wave hits.

As soon as you feel that emotional wave coming, grab an ice cube and squeeze it in your hand, or splash your face with cold water. Just focus on how cold it feels for thirty seconds.

This jolts your nervous system from emotional overload into physical awareness. It breaks the hijack long enough for your rational brain to come back online.

The pain worsens when you try to think your way out. The switch gives your brain an off-ramp.

4/4 – Brain scans show that ADHD RSD lights up the same pain centers as getting physically hurt. Your feelings aren't over the top.

They are neurologically amplified. This means that what feels like a ten to you may register as a two to someone else, not because you are weak, but because your brain processes it differently.

Insufficient evidence 4

Claim #10 · **Insufficient evidence** · Low confidence

Research Quality: Claim: people with ADHD thrive in fast-changing, high-urgency roles. No study tests this; the only evidence is expert consensus on occupational difficulty [1] and a cross-sectional burnout study [2] (GRADE very low, no consensus). This is the popular "superpower" framing without data. Occupational cohort studies comparing job types would be needed.

X Post Link · Citations for this claim

The dream ADHD career: A role where you’re allowed to jump between projects, solve unexpected problems, and work in short, intense bursts of hyperfocus… without being punished for not having a “consistent routine.”1/2

1/2 – The dream ADHD career: A role where you’re allowed to jump between projects, solve unexpected problems, and work in short, intense bursts of hyperfocus… without being punished for not having a “consistent routine.”1/2

2/2 – We thrive in chaos and variety.
Give us tight deadlines, changing priorities, and the freedom to do things our own way, and we can move mountains.
But put us in a slow, repetitive 9-5 with the same task every day? Our brain checks out and starts planning an escape.

We're not undisciplined.
We're high-performance engines that run best on novelty and urgency. 2/2

Claim #11 · Insufficient evidence · Low confidence

Research Quality: Claim: removing setup steps and body doubling help people with ADHD start tasks. Neither has been tested directly. Environmental restructuring sits inside CBT packages that work [1], and the only body-doubling test (EEG pilot, N=26) found no ADHD-specific effect [2] (GRADE very low). Red flag: a small, underpowered conference paper; rCTs with task-initiation outcomes would strengthen it.

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5/7 – So get the steps down to zero the night before. Guitar out of the case and on the couch. Paints open on the table. The game already loaded.
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Claim #14 · Insufficient evidence · Low confidence

Research Quality: Claim: cold-water shock, saying "this is real pain", and a 20-minute pause calm ADHD emotional waves. These techniques resemble DBT skills, but the only RCT of a DBT-based package in adult ADHD (n=121, pre-registered) did not improve emotion regulation at 14 weeks [1] (GRADE very low). Red flags: an unblinded trial with self-reported outcomes (RoB 2: some concerns), and the evidence is about the package, not these tips. Dismantling trials testing each skill would be needed.

X Post Link · X Post Link 2 · X Post Link 3 · Citations for this claim

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They are neurologically amplified. This means that what feels like a ten to you may register as a two to someone else, not because you are weak, but because your brain processes it differently.

ALSO CITED: ADHD RESEARCH PROVEN THAT ADHD BRAINS REGISTER SMALL COMMENTS AND TINY CRITICISM THE SAME WAY A NEUROTYPICAL BRAIN REGISTERS PHYSICAL PAIN: 1/4

1/4 – ADHD research proven that ADHD brains register small comments and tiny criticism the same way a neurotypical brain registers PHYSICAL PAIN: 1/4

2/4 – Someone sighs, corrects you, or just sounds annoyed. Suddenly, it feels like you got punched in the gut.

This happens because your dorsal anterior cingulate cortex, the brain region that processes physical pain,

fires at full force. Neurotypical brains filter these minor signals and discard them. Yours cannot.

People say, “Just ignore it.” But your brain is yelling, “I'm hurt!” You can't just think your way out of it, any more than you could just wish away a broken arm.

3/4 – Use “Pain Is Real” response.
When you feel that sting, put your hand where it hurts - usually your chest or stomach - and say out loud, “My brain thinks this is real pain. I'm not making it up. I'm actually hurt.”

Then treat yourself accordingly, with rest, cold water, or a quiet space.

The struggle worsens when you dismiss your own pain as weakness.
Validating it stops the shame spiral that follows every ADHD RSD episode and allows your brain to begin calming.

4/4 – RSD isn’t just a figure of speech.

Brain scans show that the anterior cingulate cortex, the same area that activates during physical trauma, lights up when an ADHD brain perceives rejection.

Emotional pain is physical pain for the ADHD brain, and it needs real care, not dismissal.

ALSO CITED: **STOP CALLING YOURSELF "TOO SENSITIVE." THAT UNANSWERED TEXT DIDN'T HURT BECAUSE YOU'RE DRAMATIC. YOUR ADHD BRAIN FEELS REJECTION AT A DIFFERENT VOLUME.**

1/6 – Stop calling yourself "too sensitive." That unanswered text didn't hurt because you're dramatic. Your ADHD brain feels rejection at a different volume.

They replied "ok." No emoji, no exclamation mark. Now you've spent two hours rewriting everything you might have done wrong. That's rejection sensitivity.

2/6 – 1. Rejection sensitive dysphoria (RSD) is an intense emotional pain triggered by perceived rejection, criticism, or disappointing someone. The key word is "perceived." It can hit even when nobody actually rejected you.

3/6 – 2. It's not weakness. Emotional dysregulation is common in ADHD, so feelings arrive faster and stronger, and they take longer to settle. Many people with ADHD also spend years hearing criticism, which can make the brain braced for it.

4/6 – 3. Common signs: replaying conversations for hours, assuming silence means anger, people-pleasing to avoid disapproval, avoiding things you might fail at, or going from fine to devastated in seconds over a tone or a look.

5/6 – 4. Pause before you react. When the wave hits, wait 20 minutes before replying, quitting, or apologizing. The first story your brain tells is usually the harshest one, and the feeling drops faster than you'd expect.

6/6 – 5. Check the story, not just the feeling. Ask: "What do I actually know?" versus "What am I assuming?" Then tell the people close to you how it works, like "If I seem distant after feedback, I'm processing, not angry." It takes pressure off both sides.

Claim #18 · Insufficient evidence · Low confidence

Research Quality: Claim: most people with ADHD never ask for accommodations and burn out quietly. Qualitative interviews describe disclosure dilemmas [1], and a non-peer-reviewed advocacy survey suggests about half of neurodivergent workers have asked [2] (GRADE very low, no data). Red flags: grey literature, a self-selected sample, and no ADHD-specific data. A representative workforce survey of adults with diagnosed ADHD would be needed.

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A ramp isn't a special favor for a wheelchair user. It's equal access. ADHD accommodations work exactly the same way.

6/7 – 5. Without them, the job costs double.

The harder the work, the more these matter. Without them, half your energy goes into fighting the environment instead of doing the work.
That gap is where burnout comes from.

7/7 – So the headphones aren't a favor, and the written recap isn't a weakness. They're what lets the work you're actually good at show up.
Some managers will still roll their eyes. And asking can still feel like admitting you can't keep up. It isn't. It's the reason you can.

Unsupported 6

Claim #1 · Unsupported · High confidence

Research Quality: Claim: masking is the single biggest predictor of ADHD burnout, ahead of workload, and it peaks in the late 20s to early 30s. The best evidence is cross-sectional (GRADE very low), and there is no research consensus. Adults with ADHD do camouflage more [1], but no study ranks burnout predictors, and the only ADHD job-burnout study points to executive-function demands instead [2]. Red flags: a "BREAKING" claim with no study named, and an autism-specific camouflaging scale; a prospective cohort with a validated burnout measure would strengthen it.

X Post Link · Citations for this claim

BREAKING: ADHD researchers confirm the single biggest predictor of burnout in adults with ADHD.

1/6 – BREAKING: ADHD researchers confirm the single biggest predictor of burnout in adults with ADHD.

It's not workload.

It's spending 30 years pretending to be neurotypical. 1/6

2/6 – 1. Masking is not passive. It is an active, effortful performance that runs in the background of every single interaction. Appear interested. Do not interrupt. Sit still. Make the right amount of eye contact. Remember what they said. Do not make it weird. Every conversation costs more than it looks like it should. 2/6

3/6 – 2. The energy required to mask adds up. Day after day. Year after year. The bill does not arrive slowly. It arrives all at once. Usually in your late 20s or early 30s when the accumulated debt of performing neurotypicality finally exceeds what the system can sustain. 3/6

4/6 – 3. ADHD burnout does not look like regular burnout. It is not just tiredness. It is a complete system shutdown. The strategies that held everything together stop working. Things that were manageable with effort become impossible. The person who was always fine is suddenly not fine at all and cannot explain what changed. 4/6

5/6 – 4. What changed is not the person. What changed is that the mask finally ran out of fuel. The exhaustion that was always there but being managed through pure force of will stopped being manageable. The tank that was always running too low finally hit empty. 5/6

6/6 – 5. You were not managing fine. You were managing with a hidden cost that nobody was counting. The burnout is not a failure. It is a receipt. For years of performing a version of yourself your brain was never built to sustain. 6/6

Claim #2 · Unsupported (contradicted) · High confidence

Research Quality: Claim: ADHD burnout and depression need opposite treatments, and CBT and antidepressants fail. Level 1 evidence (Cochrane review [1], an RCT meta-analysis [2], and a BMJ umbrella review [3]) shows CBT helps adults with ADHD (SMD 0.53, moderate certainty), so the claim is contradicted; adding CBT to medication also helps in the short term [4]. There is a grain of truth: undiagnosed ADHD can be missed when only depression is treated [5]. Red flags: unblinded therapy trials, and one Cochrane author declared pharma speaker fees; trials showing CBT failing in a defined "ADHD burnout" group would be needed.

X Post Link · Citations for this claim

YOUR THERAPIST THINKS YOU'RE DEPRESSED.

1/4 – YOUR THERAPIST THINKS YOU'RE DEPRESSED.

You might be in ADHD burnout. They look identical from the outside. They need opposite treatments. Getting it wrong costs years. 1/4

2/4 – 1. Depression says you cannot feel joy. ADHD burnout says you can feel joy, but you cannot start, choose, or finish. Both bring exhaustion, sleep chaos, irritability, and hopelessness.

The difference lives in the pattern. With depression, energy often returns after rest. With ADHD burnout, rest does not restore you because your brain is still running twelve tabs and masking every one. 2/4

3/4 – 2. Anxiety can look like ADHD: racing thoughts, restlessness, doom. But ADHD restlessness is often sensory or understimulation, not fear. Borderline traits can look like rejection sensitivity and mood storms. Autistic burnout can look like ADHD shutdown, but the cause is masking and sensory overload.

Misdiagnosis happens when clinicians treat the visible emotion, not the nervous system below. 3/4

4/4 – 3. Standard CBT can fail when the problem is executive function, not distorted thinking. Antidepressants can fail when the problem is dopamine, not serotonin. Relief comes from accurate recognition: ADHD coaching, medication, sensory accommodation, nervous system safety, and grief for lost years.

You are not lazy, broken, or treatment resistant. You were described with the wrong map. 4/4

Claim #4 · Unsupported · High confidence

Research Quality: Claim: neuroscientists have confirmed shame is the #1 hidden symptom of ADHD. Shame is not a diagnostic symptom, and no study ranks it. A Level 1 review of cross-sectional studies (GRADE low) shows only that low self-esteem is common [1]. Red flags: "BREAKING" framing with no source, and poor

confounder control in the underlying studies; validated shame measures in large ADHD cohorts would be needed.

X Post Link · Citations for this claim

BREAKING: Neuroscientists confirm that shame is the number 1 hidden symptom of ADHD.

1/6 – BREAKING: Neuroscientists confirm that shame is the number 1 hidden symptom of ADHD.

Not hyperactivity. Not distraction.

Shame.

From a lifetime of being told you're not trying hard enough. 1/6

2/6 – 1. Every person with undiagnosed ADHD carries a running list of evidence that they are fundamentally not enough. The forgotten things. The unfinished things. The broken promises. The wasted opportunities. The list has been building since childhood and they believe every single item on it. 2/6

3/6 – 2. The shame is not a side effect of ADHD. It is a symptom. It is what happens when a neurological condition goes unnamed and every manifestation of it gets explained as a character flaw instead. You are not lazy. But you have been told you are lazy for so long that the difference stopped mattering. 3/6

4/6 – 3. Shame in ADHD compounds everything. It is not just an emotion. It is a neurological problem. Shame floods the prefrontal cortex with cortisol. The prefrontal cortex is also where executive function lives. Shame actively destroys the function it is blaming you for not having. 4/6

5/6 – 4. This is why self compassion is not soft. For someone with ADHD, reducing shame is neurologically strategic. Less shame means less cortisol. Less cortisol means better prefrontal access. Better prefrontal access means better executive function. Kindness toward yourself is literally functional. 5/6

6/6 – 5. The shame was never accurate. It was the only explanation available when the real one was missing. Now there is a real one. And the real one has nothing to do with effort and everything to do with neurology. 6/6

Claim #7 · Unsupported · High confidence

Research Quality: Claim: RSD is a distinct condition that is neurological, not emotional. Rejection sensitivity is higher with ADHD symptoms in large cross-sectional samples [1] [2] (Level 5, GRADE low), but RSD is not in DSM-5-TR or ICD-11, and its first systematic review is still only a protocol [3]. The verdict turns on the contested consensus and the false neurological-vs-emotional split. Red flag: the direct evidence is a 5-person qualitative study, which has a correction notice [4]; a validated RSD measure showing it is distinct from emotion dysregulation would be needed.

X Post Link · X Post Link 2 · Citations for this claim

Stop calling yourself "too sensitive." That unanswered text didn't hurt because you're dramatic. Your ADHD brain feels rejection at a different volume.

1/6 – Stop calling yourself "too sensitive." That unanswered text didn't hurt because you're dramatic. Your ADHD brain feels rejection at a different volume.

They replied "ok." No emoji, no exclamation mark. Now you've spent two hours rewriting everything you might have done wrong. That's rejection sensitivity.

2/6 – 1. Rejection sensitive dysphoria (RSD) is an intense emotional pain triggered by perceived rejection, criticism, or disappointing someone. The key word is "perceived." It can hit even when nobody actually rejected you.

3/6 – 2. It's not weakness. Emotional dysregulation is common in ADHD, so feelings arrive faster and stronger, and they take longer to settle. Many people with ADHD also spend years hearing criticism, which can make the brain braced for it.

4/6 – 3. Common signs: replaying conversations for hours, assuming silence means anger, people-pleasing to avoid disapproval, avoiding things you might fail at, or going from fine to devastated in seconds over a tone or a look.

5/6 – 4. Pause before you react. When the wave hits, wait 20 minutes before replying, quitting, or apologizing. The first story your brain tells is usually the harshest one, and the feeling drops faster than you'd expect.

6/6 – 5. Check the story, not just the feeling. Ask: "What do I actually know?" versus "What am I assuming?" Then tell the people close to you how it works, like "If I seem distant after feedback, I'm processing, not angry." It takes pressure off both sides.

ALSO CITED: WAIT WAIT WAIT SO APPARENTLY IF YOU'VE BEEN CALLED "TOO SENSITIVE YOUR WHOLE LIFE" BUT YOU ACTUALLY HAVE ADHD THAT SENSITIVITY HAS A NAME.

1/6 – WAIT WAIT WAIT so apparently if you've been called "too sensitive your whole life" but you actually have ADHD that sensitivity has a name.

It's called Rejection Sensitive Dysphoria.

And it is neurological, not emotional. 1/6

2/6 – 1. RSD means your brain physically processes rejection the same way it processes physical pain. A harsh tone. A short reply. Someone cancelling plans. Your nervous system is not overreacting. It is reporting accurately on what it is experiencing. 2/6

3/6 – 2. The intensity is not proportionate to the event because it was never about the event. It is about a nervous system that has no volume dial on emotional pain. The criticism that takes a neurotypical person two minutes to shake takes an ADHD brain hours. Sometimes days. 3/6

4/6 – 3. This is why people with ADHD often become people pleasers, perfectionists, or people who simply stop trying. Not weakness. Risk management. If you never put yourself out there you cannot be rejected. If you are perfect there is nothing to criticise. The avoidance makes complete sense. 4/6

5/6 – 4. The shame spiral after an emotional reaction makes it worse. You felt it intensely. Then you felt ashamed of feeling it intensely. Then you felt ashamed of the shame. The ADHD brain stacks emotional weight fast and has no automatic system for putting it back down. 5/6

6/6 – 5. You were never too sensitive. You had a nervous system running a pain response without a label. The people who called you too much were describing a symptom they did not have the language to name.

You have the language now. It changes the whole story. 6/6

Claim #8 · **Unsupported (contradicted)** · High confidence

Research Quality: Claim: brain scans show small criticism lights up the same pain centres as physical injury in ADHD. A non-ADHD voxel meta-analysis shows anterior-cingulate activity during social pain [1], but multivariate fMRI finds social rejection and physical pain are represented separately [2], which contradicts the "same pain" claim (GRADE very low, contested). We found no ADHD-specific imaging study. Red flags: "brain scans prove" with no source, and indirectness; aDHD vs control fMRI with pattern-level analysis would be needed.

X Post Link · X Post Link 2 · X Post Link 3 · Citations for this claim

ADHD research proven that ADHD brains register small comments and tiny criticism the same way a neurotypical brain registers PHYSICAL PAIN: 1/4

1/4 – ADHD research proven that ADHD brains register small comments and tiny criticism the same way a neurotypical brain registers PHYSICAL PAIN: 1/4

2/4 – Someone sighs, corrects you, or just sounds annoyed. Suddenly, it feels like you got punched in the gut.

This happens because your dorsal anterior cingulate cortex, the brain region that processes physical pain, fires at full force. Neurotypical brains filter these minor signals and discard them. Yours cannot.

People say, “Just ignore it.” But your brain is yelling, “I'm hurt!” You can't just think your way out of it, any more than you could just wish away a broken arm.

3/4 – Use “Pain Is Real” response.
When you feel that sting, put your hand where it hurts - usually your chest or stomach - and say out loud, “My brain thinks this is real pain. I'm not making it up. I'm actually hurt.”

Then treat yourself accordingly, with rest, cold water, or a quiet space.

The struggle worsens when you dismiss your own pain as weakness.
Validating it stops the shame spiral that follows every ADHD RSD episode and allows your brain to begin calming.

4/4 – RSD isn’t just a figure of speech.

Brain scans show that the anterior cingulate cortex, the same area that activates during physical trauma, lights up when an ADHD brain perceives rejection.

Emotional pain is physical pain for the ADHD brain, and it needs real care, not dismissal.

ALSO CITED: WAIT WAIT WAIT SO APPARENTLY IF YOU'VE BEEN CALLED "TOO SENSITIVE YOUR WHOLE LIFE" BUT YOU ACTUALLY HAVE ADHD THAT SENSITIVITY HAS A NAME.

1/6 – WAIT WAIT WAIT so apparently if you've been called "too sensitive your whole life" but you actually have ADHD that sensitivity has a name.

It's called Rejection Sensitive Dysphoria.

And it is neurological, not emotional. 1/6

2/6 – 1. RSD means your brain physically processes rejection the same way it processes physical pain. A harsh tone. A short reply. Someone cancelling plans. Your nervous system is not overreacting. It is reporting accurately on what it is experiencing. 2/6

3/6 – 2. The intensity is not proportionate to the event because it was never about the event. It is about a nervous system that has no volume dial on emotional pain. The criticism that takes a neurotypical person two minutes to shake takes an ADHD brain hours. Sometimes days. 3/6

4/6 – 3. This is why people with ADHD often become people pleasers, perfectionists, or people who simply stop trying. Not weakness. Risk management. If you never put yourself out there you cannot be rejected. If you are perfect there is nothing to criticise. The avoidance makes complete sense. 4/6

5/6 – 4. The shame spiral after an emotional reaction makes it worse. You felt it intensely. Then you felt ashamed of feeling it intensely. Then you felt ashamed of the shame. The ADHD brain stacks emotional weight fast and has no automatic system for putting it back down. 5/6

6/6 – 5. You were never too sensitive. You had a nervous system running a pain response without a label. The people who called you too much were describing a symptom they did not have the language to name.

You have the language now. It changes the whole story. 6/6

ALSO CITED: **BRAIN SCANS PROVED WHY EMOTIONS HIT AT 100% FOR PEOPLE WITH ADHD, AND YOU CAN'T STOP THEM. YOU ARE NOT TOO SENSITIVE OR DRAMATIC: 1/4**

1/4 – Brain scans proved why emotions hit at 100% for people with ADHD, and you can't stop them. You are not too sensitive or dramatic: 1/4

2/4 – Your brain lacks the filter that others have.
One comment, someone forgetting to text back, or even a weird look can hit you all at once. People have probably told you forever that you overreact, need to toughen up, or make things a bigger deal than they are.

But brain scans show your amygdala fires like a real threat is in front of you. You cannot simply decide to feel less. Trying to suppress it only makes the pressure build until you break down or lash out, and the shame cycle begins again.

3/4 – Use a “Sensory Shock Switch” when the wave hits.

As soon as you feel that emotional wave coming, grab an ice cube and squeeze it in your hand, or splash your face with cold water. Just focus on how cold it feels for thirty seconds.

This jolts your nervous system from emotional overload into physical awareness. It breaks the hijack long enough for your rational brain to come back online.

The pain worsens when you try to think your way out. The switch gives your brain an off-ramp.

4/4 – Brain scans show that ADHD RSD lights up the same pain centers as getting physically hurt. Your feelings aren't over the top.

They are neurologically amplified. This means that what feels like a ten to you may register as a two to someone else, not because you are weak, but because your brain processes it differently.

Claim #16 · **Unsupported** · Moderate confidence

Research Quality: Claim: brain scans show the ADHD amygdala fires like facing a real threat. A systematic review of youth fMRI studies finds altered amygdala–prefrontal connectivity [1], but one case-control study found lower, not higher, amygdala activation [2] (GRADE very low, contested). Red flags: no source cited, small samples, and inconsistent direction of findings. A coordinate-based meta-analysis would be needed.

X Post Link · Citations for this claim

Brain scans proved why emotions hit at 100% for people with ADHD, and you can't stop them. You are not too sensitive or dramatic: 1/4

1/4 – Brain scans proved why emotions hit at 100% for people with ADHD, and you can't stop them. You are not too sensitive or dramatic: 1/4

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They are neurologically amplified. This means that what feels like a ten to you may register as a two to someone else, not because you are weak, but because your brain processes it differently.

Section 2 · Citations

One group per claim, in the same order as Section 1. Numbers match the [n] markers in each claim's Research Quality note. A source cited by several claims is repeated in each group.

Claim #6 – Brain scans proved why emotions hit at 100% for people with ADHD, and you can't stop them. You are not too sensitive or dramatic: 1/4

↑ Back to claim

1. Beheshti A, Chavanon ML, Christiansen H (2020). Emotion dysregulation in adults with attention deficit hyperactivity disorder: a meta-analysis. *BMC Psychiatry*. doi.org/10.1186/s12888-020-2442-7
   Meta-analysis 13 studies, N=2,535 · PROSPERO-registered; authors report an asymmetric funnel plot (possible publication bias / small-study effects), so g=1.17 may be inflated; no COI.
2. Graziano PA, Garcia A (2016). Attention-deficit hyperactivity disorder and children's emotion dysregulation: a meta-analysis. *Clinical Psychology Review*. doi.org/10.1016/j.cpr.2016.04.011
   Meta-analysis 77 studies, n=32,044 youths · Large independent replication in youth (reactivity/lability d=0.95); effect weaker after controlling for cognitive functioning; full text not checked for bias tests.

Claim #15 – Vitamin B12 deficiency can affect your brain more than you realize. 🧠Low levels may contribute to fatigue, memory problems, poor concentration, and nerve damage.

↑ Back to claim

1. Stabler SP (2013). Clinical practice. Vitamin B12 deficiency. *New England Journal of Medicine*. doi.org/10.1056/NEJMcp1113996
   Clinical review n/a · Authoritative expert narrative review drawing on case series and treatment-response data; not a systematic review.

Claim #3 – YOUR THERAPIST THINKS YOU'RE DEPRESSED.

↑ Back to claim

1. Soler-Gutiérrez AM, Pérez-González JC, Mayas J (2023). Evidence of emotion dysregulation as a core symptom of adult ADHD: a systematic review. *PLOS ONE*. doi.org/10.1371/journal.pone.0280131
   Systematic review 22 studies · Narrative synthesis, no pooled estimate; mostly small cross-sectional studies; no COI.

Claim #5 – BREAKING: Neuroscientists confirm that shame is the number 1 hidden symptom of ADHD.

↑ Back to claim

1. Dickerson SS, Kemeny ME (2004). Acute stressors and cortisol responses: a theoretical integration and synthesis of laboratory research. *Psychological Bulletin*. doi.org/10.1037/0033-2909.130.3.355
   Meta-analysis 208 lab studies · Social-evaluative, uncontrollable stressors gave the largest cortisol responses; general population, not ADHD.
2. Shields GS, Sazma MA, Yonelinas AP (2016). The effects of acute stress on core executive functions: a meta-analysis and comparison with cortisol. *Neuroscience & Biobehavioral Reviews*. doi.org/10.1016/j.neubiorev.2016.06.038
   Meta-analysis WM: 34 studies, N=1,353 · Acute stress impaired working memory (g=−0.20, small); Egger's test non-significant after removing one outlier; cortisol administration effects differed from stress effects.
3. Arnsten AFT (2009). Stress signalling pathways that impair prefrontal cortex structure and function. *Nature Reviews Neuroscience*. doi.org/10.1038/nrn2648
   Narrative review n/a (mostly animal studies) · Mechanistic review, largely rodent and primate data; NIH-funded; extrapolation to human shame is indirect.

Claim #9 – THE CRUELEST PART OF ADHD ISN'T THE BORING STUFF. IT'S THE GUITAR IN THE CORNER THAT NEVER GETS PLAYED.

↑ Back to claim

1. Berridge KC, Robinson TE (2016). Liking, wanting, and the incentive-sensitization theory of addiction. *American Psychologist*. doi.org/10.1037/amp0000059
   Theoretical review n/a · Well-replicated framework, but built mainly on animal and addiction research, not ADHD.
2. Volkow ND, Wang GJ, Kollins SH, et al. (2009). Evaluating dopamine reward pathway in ADHD: clinical implications. *JAMA*. doi.org/10.1001/jama.2009.1308
   PET case-control 53 never-medicated ADHD vs 44 controls · Never-medicated adults (reduces drug confound); an erratum was published (JAMA 2009;302:1420); single site.
3. Volkow ND, Wang GJ, Newcorn JH, et al. (2011). Motivation deficit in ADHD is associated with dysfunction of the dopamine reward pathway. *Molecular Psychiatry*. doi.org/10.1038/mp.2010.97
   PET case-control (secondary analysis) 45 ADHD vs 41 controls · Correlational secondary analysis of the 2009 sample (r≈0.35–0.41); NIH-funded.
4. Plichta MM, Scheres A (2014). Ventral-striatal responsiveness during reward anticipation in ADHD and its relation to trait impulsivity in the healthy population: a meta-analytic review of the fMRI literature. *Neuroscience & Biobehavioral Reviews*. doi.org/10.1016/j.neubiorev.2013.07.012
   Meta-analysis (fMRI) 8 studies, N=340 · Medium effect (d=0.48); few, small studies (authors note limited power); no COI.
5. Fusar-Poli P, Rubia K, Rossi G, et al. (2012). Striatal dopamine transporter alterations in ADHD: pathophysiology or adaptation to psychostimulants? A meta-analysis. *American Journal of Psychiatry*. doi.org/10.1176/appi.ajp.2011.11060940
   Meta-analysis 9 PET/SPECT studies, 169 vs 173 · Dopamine-transporter findings inconsistent (high heterogeneity) and linked to prior stimulant exposure, which cautions against a simple 'low dopamine' story.

Claim #12 – ACTUALLY, YOU ARE ALLOWED TO ASK FOR ADHD WORKPLACE ACCOMMODATIONS...

↑ Back to claim

1. Alderson RM, Kasper LJ, Hudec KL, et al. (2013). Attention-deficit/hyperactivity disorder (ADHD) and working memory in adults: a meta-analytic review. *Neuropsychology*. doi.org/10.1037/a0032371
   Meta-analysis 38 studies · Moderate between-group effects in verbal and visuospatial WM; heterogeneity partly explained by task moderators; bias tests not verifiable (no free full text).

Claim #13 – ACTUALLY, YOU ARE ALLOWED TO ASK FOR ADHD WORKPLACE ACCOMMODATIONS...

↑ Back to claim

1. Nigg JT, Bruton A, Kozlowski MB, et al. (2024). Systematic review and meta-analysis: do white noise or pink noise help with task performance in youth with attention-deficit/hyperactivity disorder or with elevated attention problems?. *Journal of the American Academy of Child & Adolescent Psychiatry*. doi.org/10.1016/j.jaac.2023.12.014
   Meta-analysis 13 studies, n=335 · PROSPERO-registered; trim-and-fill found no publication bias; authors' GRADE moderate (downgraded for unblinded designs); lab tasks only; no COI.
2. Söderlund GBW, Hadjikhani N, Thorsson M, et al. (2024). Sensory white noise in clinical ADHD: who benefits from noise, and who performs worse?. *Scandinavian Journal of Child and Adolescent Psychiatry and Psychology*. doi.org/10.2478/sjcapp-2024-0010
   Experimental study Clinical ADHD sample (children) · No overall benefit; benefit varied by symptom profile; lead author's group originated the noise-benefit hypothesis.

Claim #17 – Brain scans proved why emotions hit at 100% for people with ADHD, and you can't stop them. You are not too sensitive or dramatic: 1/4

↑ Back to claim

1. Materna L, Wiesner CD, Shushakova A, et al. (2019). Adult patients with ADHD differ from healthy controls in implicit, but not explicit, emotion regulation. *Journal of Psychiatry & Neuroscience*. doi.org/10.1503/jpn.180139
   fMRI case-control 30 ADHD vs 35 controls · ADHD group used more habitual suppression and less reappraisal; instructed reappraisal not impaired.
2. Matthies S, Philipsen A, Lackner HK, et al. (2014). Regulation of sadness via acceptance or suppression in adult attention deficit hyperactivity disorder (ADHD). *Psychiatry Research*. doi.org/10.1016/j.psychres.2014.07.017
   Randomised lab experiment n=36 (ADHD only) · Small; no non-ADHD comparison group; short-term lab outcome.

Claim #10 – The dream ADHD career: A role where you're allowed to jump between projects, solve unexpected problems, and work in short, intense bursts of hyperfocus… without being punished for not having a "consistent routine."1/2

↑ Back to claim

1. Adamou M, Arif M, Asherson P, et al. (2013). Occupational issues of adults with ADHD. *BMC Psychiatry*. doi.org/10.1186/1471-244X-13-59
   Consensus statement Expert panel · Expert opinion (lowest evidence tier); useful context on workplace impairment.
2. Turjeman-Levi Y, Itzchakov G, Engel-Yeger B (2024). Executive function deficits mediate the relationship between employees' ADHD and job burnout. *AIMS Public Health*. doi.org/10.3934/publichealth.2024015
   Cross-sectional field study n=171 · Self-report only; mediation tested on cross-sectional data, so causal direction cannot be established (Newcastle-Ottawa: low).

Claim #11 – THE CRUELEST PART OF ADHD ISN'T THE BORING STUFF. IT'S THE GUITAR IN THE CORNER THAT NEVER GETS PLAYED.

↑ Back to claim

1. Liu CI, Hua MH, Lu ML, Goh KK (2023). Effectiveness of cognitive behavioural-based interventions for adults with attention-deficit/hyperactivity disorder extends beyond core symptoms: a meta-analysis of randomized controlled trials. *Psychology and Psychotherapy: Theory, Research and Practice*. doi.org/10.1111/papt.12455
   Meta-analysis of RCTs 28 RCTs · PROSPERO-registered (CRD42021273633); heterogeneous control conditions.
2. Schuenke R, Dickenson SR, Moore M (2025). Reading between the lines: exploring body doubling in ADHD using EEG. *Proceedings of ACM ASSETS 2025*. doi.org/10.1145/3663547.3759743
   Pilot experiment N=26 · Conference paper; small, underpowered; no significant ADHD-specific effect.

Claim #14 – Brain scans proved why emotions hit at 100% for people with ADHD, and you can't stop them. You are not too sensitive or dramatic: 1/4

↑ Back to claim

1. Halmøy A, Ring AE, Gjestad R, et al. (2022). Dialectical behavioral therapy-based group treatment versus treatment as usual for adults with attention-deficit hyperactivity disorder: a multicenter randomized controlled trial. *BMC Psychiatry*. doi.org/10.1186/s12888-022-04356-6
   RCT n=121 · Pre-registered (ISRCTN30469893, NCT02685254); unblinded with self-reported outcomes (RoB 2: some concerns); emotion-regulation outcome not significant at 14 weeks.

Claim #18 – ACTUALLY, YOU ARE ALLOWED TO ASK FOR ADHD WORKPLACE ACCOMMODATIONS...

↑ Back to claim

1. Oscarsson M, Nelson M, Rozental A, et al. (2022). Stress and work-related mental illness among working adults with ADHD: a qualitative study. *BMC Psychiatry*. doi.org/10.1186/s12888-022-04409-w
   Qualitative interviews n=20 · Describes disclosure dilemmas; cannot estimate prevalence.
2. Understood.org / The Harris Poll (2025). 2025 "Neurodiversity at Work" survey (press release). *Understood.org*. www.understood.org/en/press-releases/2025-neurodiversity-at-work-survey
   Online survey (grey literature) 539 employed neurodivergent adults · Not peer-reviewed; self-identified neurodivergence, not ADHD-specific; advocacy organisation.

Claim #1 – BREAKING: ADHD researchers confirm the single biggest predictor of burnout in adults with ADHD.

↑ Back to claim

1. van der Putten WJ, Mol AJJ, Groenman AP, et al. (2024). Is camouflaging unique for autism? A comparison of camouflaging between adults with autism and ADHD. *Autism Research*. doi.org/10.1002/aur.3099
   Cross-sectional (pre-registered) 105 per group; 477 total · Pre-registered (AsPredicted #41811); uses an autism-specific camouflaging scale; did not measure burnout.
2. Turjeman-Levi Y, Itzchakov G, Engel-Yeger B (2024). Executive function deficits mediate the relationship between employees' ADHD and job burnout. *AIMS Public Health*. doi.org/10.3934/publichealth.2024015
   Cross-sectional field study n=171 · Self-report only; mediation tested on cross-sectional data, so causal direction cannot be established (Newcastle-Ottawa: low).

Claim #2 – YOUR THERAPIST THINKS YOU'RE DEPRESSED.

↑ Back to claim

1. Lopez PL, Torrente FM, Ciapponi A, et al. (2018). Cognitive-behavioural interventions for attention deficit hyperactivity disorder (ADHD) in adults. *Cochrane Database of Systematic Reviews*. doi.org/10.1002/14651858.CD010840.pub2
   Cochrane systematic review 14 RCTs, 700 participants · Cochrane methods with GRADE; evidence mostly low to moderate quality (unblinded therapy trials); one author declared pharma speaker fees.
2. Liu CI, Hua MH, Lu ML, Goh KK (2023). Effectiveness of cognitive behavioural-based interventions for adults with attention-deficit/hyperactivity disorder extends beyond core symptoms: a meta-analysis of randomized controlled trials. *Psychology and Psychotherapy: Theory, Research and Practice*. doi.org/10.1111/papt.12455
   Meta-analysis of RCTs 28 RCTs · PROSPERO-registered (CRD42021273633); heterogeneous control conditions.
3. Gosling CJ, Garcia-Argibay M, De Prisco M, et al. (2025). Benefits and harms of ADHD interventions: umbrella review and platform for shared decision making. *BMJ*. doi.org/10.1136/bmj-2025-085875
   Umbrella review 221 re-estimated meta-analyses · Pre-registered (OSF); AMSTAR-2 plus algorithmic GRADE; adult CBT SMD 0.53, moderate certainty; several authors declare industry ties unrelated to this work.
4. Li Y, Zhang L (2024). Efficacy of cognitive behavioral therapy combined with pharmacotherapy versus pharmacotherapy alone in adult ADHD: a systematic review and meta-analysis. *Journal of Attention Disorders*. doi.org/10.1177/10870547231214969
   Meta-analysis of RCTs 6 RCTs · Small; benefit at 3 months, not at 6–9 months.
5. Fu X, Wu W, Wu Y, et al. (2025). Adult ADHD and comorbid anxiety and depressive disorders: a review of etiology and treatment. *Frontiers in Psychiatry*. doi.org/10.3389/fpsyt.2025.1597559
   Narrative review n/a · Non-systematic; useful for comorbidity context only.

Claim #4 – BREAKING: Neuroscientists confirm that shame is the number 1 hidden symptom of ADHD.

↑ Back to claim

1. Pedersen AB, Edvardsen BV, Messina SM, et al. (2024). Self-esteem in adults with ADHD using the Rosenberg Self-Esteem Scale: a systematic review. *Journal of Attention Disorders*. doi.org/10.1177/10870547241237245
   Systematic review 11 studies · Cross-sectional studies, little confounder control; no pooled estimate; no COI.

Claim #7 – Stop calling yourself "too sensitive." That unanswered text didn't hurt because you're dramatic. Your ADHD brain feels rejection at a different volume.

↑ Back to claim

1. Bondü R, Esser G (2015). Justice and rejection sensitivity in children and adolescents with ADHD symptoms. *European Child & Adolescent Psychiatry*. doi.org/10.1007/s00787-014-0560-9
   Cross-sectional survey N=1,235 adolescents · Large community sample; self-reported symptoms; higher anxious and angry rejection sensitivity with ADHD symptoms.
2. Babinski DE, Kujawa A, Kessel EM, et al. (2019). Sensitivity to peer feedback in young adolescents with symptoms of ADHD: examination of neurophysiological and self-report measures. *Journal of Abnormal Child Psychology*. doi.org/10.1007/s10802-018-0470-2
   Cross-sectional (ERP + self-report) N=391 community adolescents · Dimensional ADHD symptoms (not diagnosis); greater symptoms linked to enhanced early ERP to rejection; NIH-funded.
3. PROSPERO registration CRD420250638010 (2025). Children and adults with ADHD experiences of rejection sensitive dysphoria (RSD): a systematic review and narrative analysis (protocol). *PROSPERO, University of York*. www.crd.york.ac.uk/PROSPERO/view/CRD420250638010
   Review protocol n/a · Registered protocol, no results yet; confirms RSD is not in DSM-5 or ICD-11.
4. Rowney-Smith A, Sutton B, Quadt L, et al. (2026). The lived experience of rejection sensitivity in ADHD: a qualitative exploration. *PLOS ONE*. doi.org/10.1371/journal.pone.0314669
   Qualitative focus groups n=5 undergraduates · Very small; hypothesis-generating only; a correction notice was published (PLOS ONE, 13 Mar 2026), not a retraction.

Claim #8 – ADHD research proven that ADHD brains register small comments and tiny criticism the same way a neurotypical brain registers PHYSICAL PAIN: 1/4

↑ Back to claim

1. Rotge JY, Lemogne C, Hinfray S, et al. (2015). A meta-analysis of the anterior cingulate contribution to social pain. *Social Cognitive and Affective Neuroscience*. doi.org/10.1093/scan/nsu110
   Meta-analysis (voxel-based fMRI) 46 studies, 940 subjects · Confirms ACC activation during social pain at the gross level (non-ADHD samples); cannot show 'same as physical pain'.
2. Woo CW, Koban L, Kross E, et al. (2014). Separate neural representations for physical pain and social rejection. *Nature Communications*. doi.org/10.1038/ncomms6380
   fMRI (multivariate) n=60 + 91 · Out-of-sample classifier validation; non-ADHD sample; NIH-funded.

Claim #16 – Brain scans proved why emotions hit at 100% for people with ADHD, and you can't stop them. You are not too sensitive or dramatic: 1/4

↑ Back to claim

1. Musella KE, Weyandt LL (2023). Attention-deficit hyperactivity disorder and youth's emotion dysregulation: a systematic review of fMRI studies. *Applied Neuropsychology: Child*. doi.org/10.1080/21622965.2022.2119142
   Systematic review 13 fMRI studies · No pooled effect; heterogeneous regions and tasks; youth only.
2. Viering T, Naaijen J, van Rooij D, et al. (2022). Amygdala reactivity and ventromedial prefrontal cortex coupling in the processing of emotional face stimuli in attention-deficit/hyperactivity disorder. *European Child & Adolescent Psychiatry*. doi.org/10.1007/s00787-021-01809-3
   fMRI case-control 61 ADHD vs 51 controls · Found lower (not higher) amygdala activation to emotional faces; single study.

About this report

18 claims re-evaluated. The attention-grabbing headlines (“BREAKING”, “brain scans prove”, “researchers confirm”) were the least reliable part: none held up. Two verdicts changed (claims 11 and 12), both because the practical tip itself has never been tested. The second run (8:06 AM) is appended to the Markdown report and has its own page.

How claims were evaluated. Evidence hierarchy: Systematic reviews and meta-analyses (Level 1), RCTs (2), cohort (3), case-control (4), cross-sectional (5), case reports and expert opinion (6). Guidelines and clinical reviews are named as such. **GRADE certainty:** Our rating of the body of evidence for this specific claim: randomised evidence starts High and observational starts Low. Ratings go down for risk of bias, inconsistency, indirectness (e.g. non-ADHD or lab-only samples), imprecision and publication bias, and up for large effects. **Verdicts:** Supported: strong, consistent, high-quality evidence. Partially supported: some solid evidence, but the claim overgeneralises or has important caveats. Insufficient evidence: too little good-quality research to judge. Unsupported: evidence is absent for the claim as stated, or contradicts it (flagged as contradicted). Confidence is our confidence in the verdict, which can be high even when GRADE is low (e.g. a well-hedged claim).

Verdict changes: 2 changed in substance; 11 relabelled onto the four-category scale with no change in substance (Not supported or Contradicted → Unsupported; Partly → Partially supported).

Claim	Previous	Now	GRADE
#11	Partly / insufficient	Insufficient evidence	Very low
#12	Supported	Partially supported	Low

Method. Posts were pulled with the X API. Self-reply parts were reconstructed via conversation_id and reply links and quoted verbatim, numbered in order. Reference metadata, corrections and retractions were checked against Crossref and PubMed, and open-access full texts were read. GRADE ratings are our own per-claim judgement unless stated.

Not medical advice. Verdicts reflect evidence as of the check date and are group-level. Talk to a qualified clinician about diagnosis or treatment.